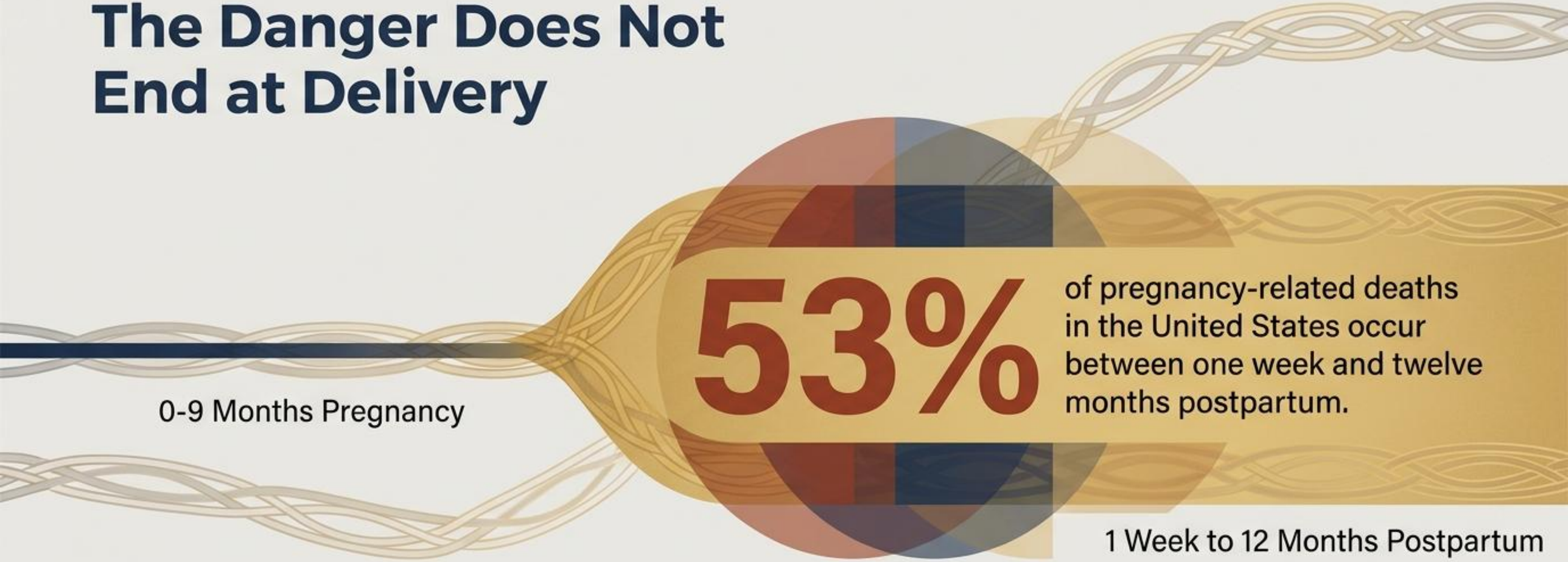
Promoting Maternal Health in the Postpartum Period to Advance Birth Equity

Insights and Structural Interventions from the Boston Black Birthing People Study

Colette Dieujuste RN, PhD
Wheaton College

wheaton college massachusetts

The Danger Does Not End at Delivery

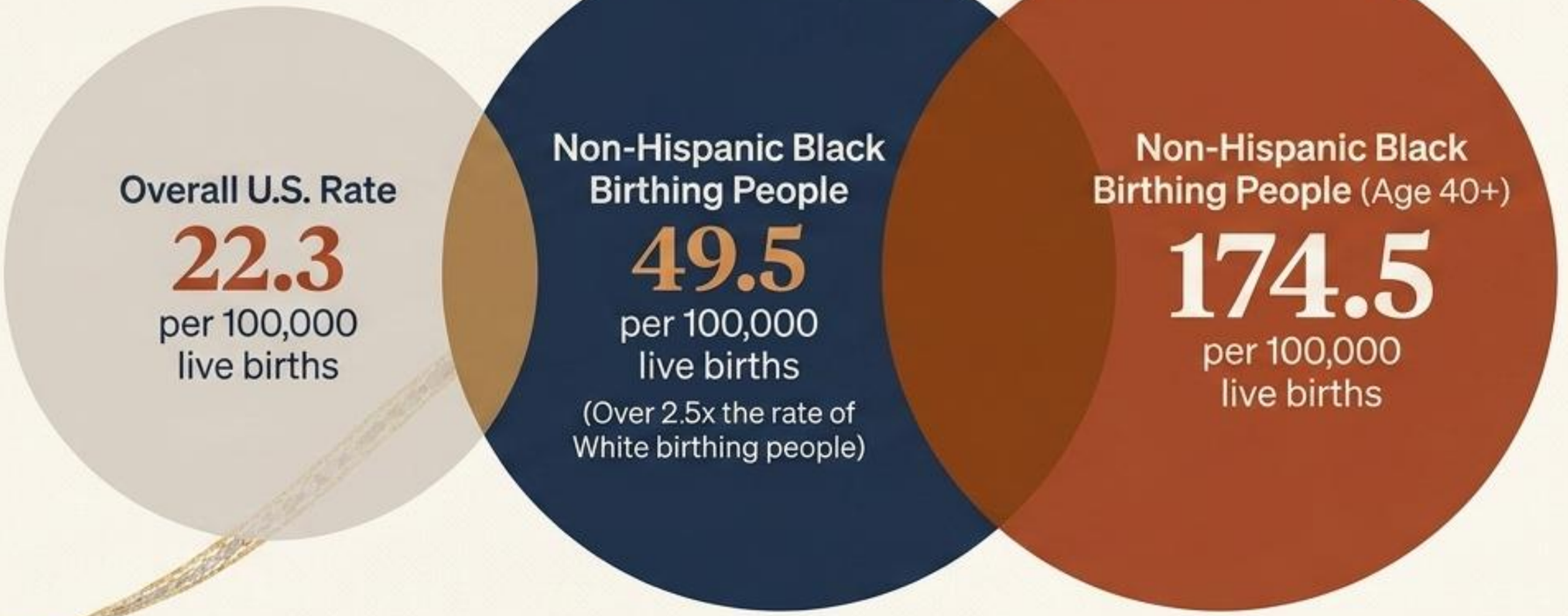


The Fourth Trimester is the critical window for identifying and treating underlying cardiovascular comorbidities, postpartum depression, and hypertensive disorders.

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A Crisis of Inequity

Maternal mortality is not distributed equally across populations.



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METHODOLOGY & SAMPLE

Cross-Sectional Survey (n= 121)

Black birthing people in the Greater Boston area who delivered within the last 24 months.

Instruments Used:

- JHPCSM: Contextualized Stress (68 items)
- PKPCT: Power and Participation (12 items)
- Demographic & Socioeconomic questionnaire



Blueprint of the Boston Study

Demographics

Assessing socioeconomic and demographic characteristics tied to postpartum follow-up attendance.

Theoretical Lenses

Examining the unseen roles of race/gender-race/gender-specific stress (Critical Race Theory) and personal agency on care utilization.

Tangible Barriers

Identifying logistical and systemic obstacles to access, specifically during the COVID-19 pandemic.

Postpartum Care Utilization

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Measuring the Invisible: The Instruments

Quantifying social determinants to correlate with clinical outcomes.

JHPCSM

Jackson, Hogue, Phillips
Contextualized Stress Measure



Developed specifically for African American women to measure 'Gendered Racial Stress'. Allows correlation of physiological stress with attendance.

PCKPCT

Barrett's Power as Knowing Participation in Change Tool

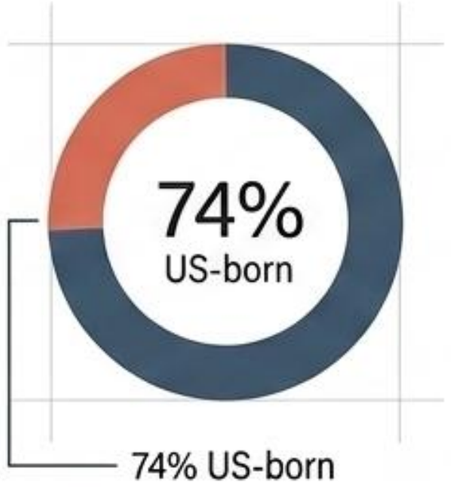


Measures 'Agency' and 'Perceived Power'.

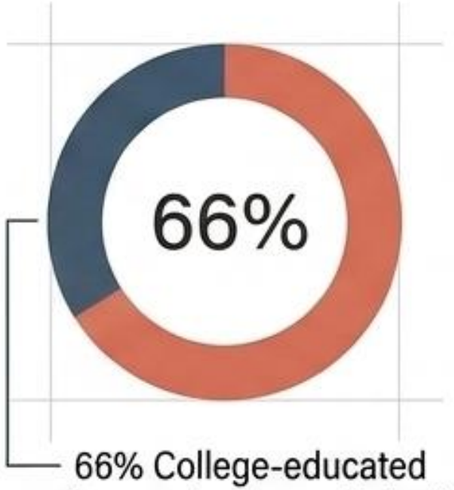
Evaluates the patient's perceived ability to participate in change.

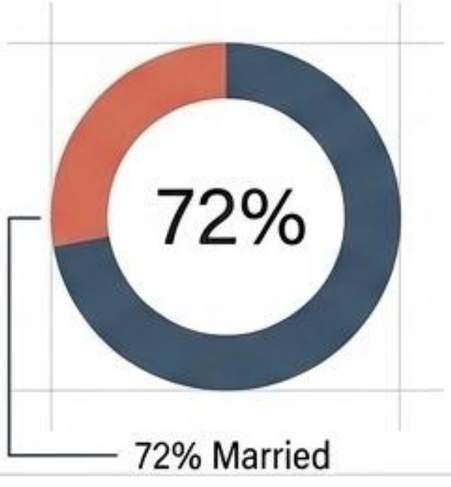
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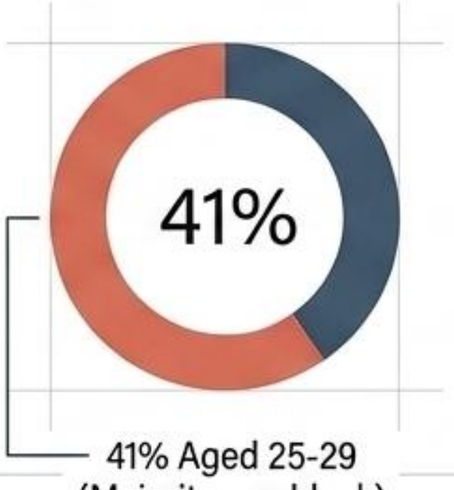
The Boston Cohort: A Demographic Snapshot



121 self-identified Black birthing people in the Greater Boston area. Data collected April 2021–January 2023 (Cross-sectional design).

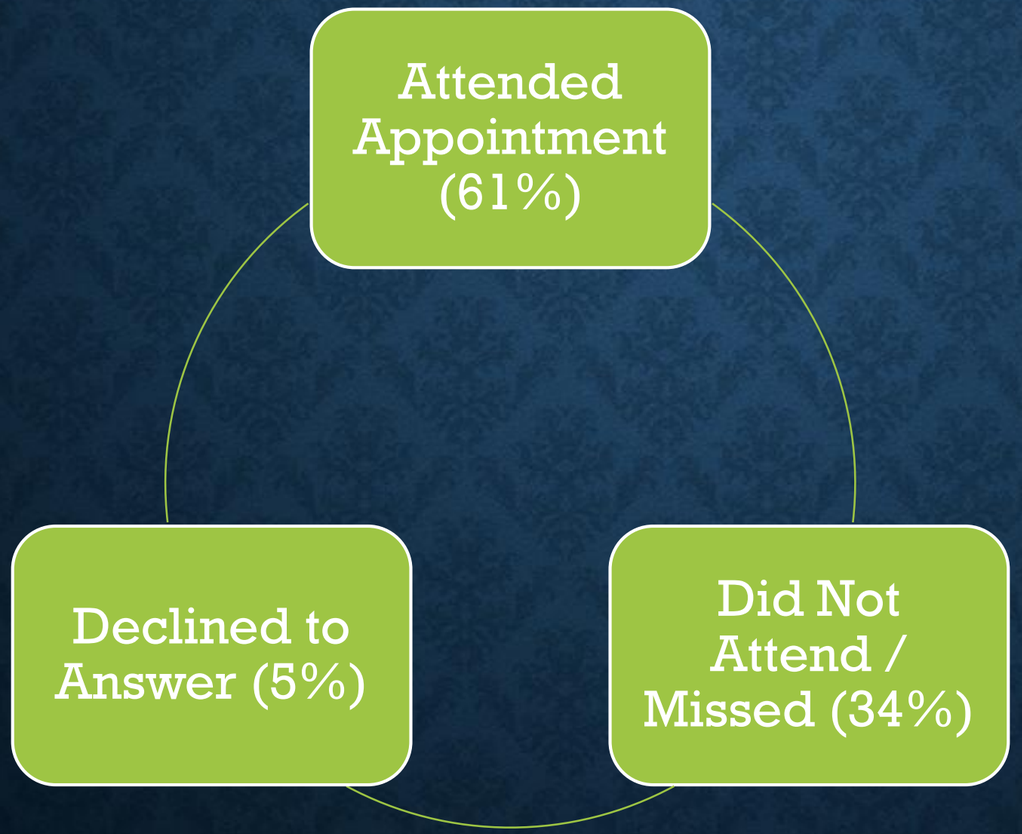






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POSTPARTUM FOLLOW-UP CARE STATUS





Attendance rate is significantly below the Healthy People 2020 goal of 90.8%.

The Attendance Divide: Economic Barriers

1 in 3

33% of participants did not attend their postpartum appointments.

Employment Constraint

Attendance was significantly lower for those working outside the home ($p < 0.001$).

Insurance Gap

Attendance was significantly associated with having private insurance ($p < 0.05$).

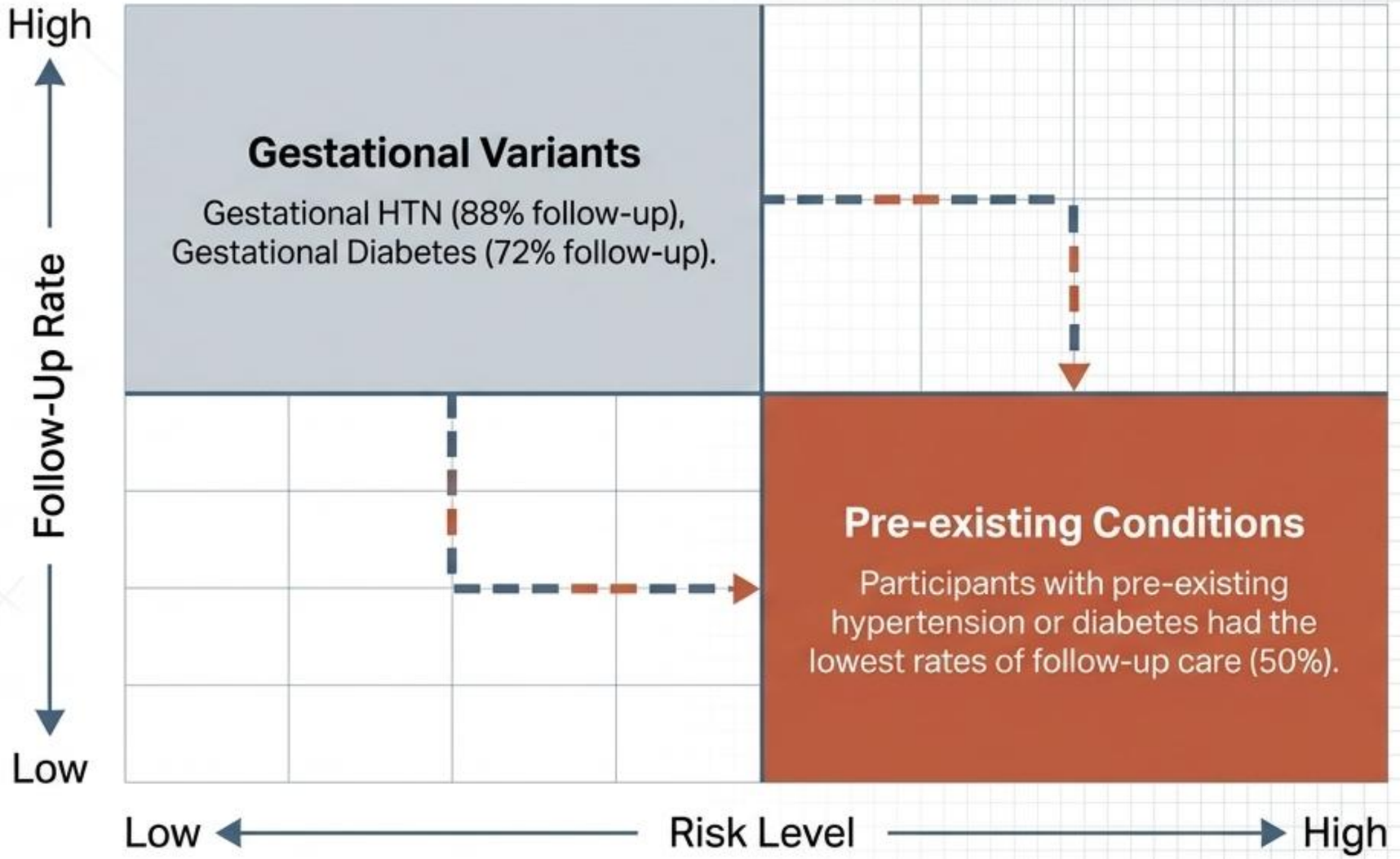
Economic survival (the need to work physically) competes directly with health survival.

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The High-Risk Disconnect

34%

Overall, 34% of participants missed their postpartum follow-up appointment.



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Socioeconomic Drivers of Care

Higher Attendance Likelihood

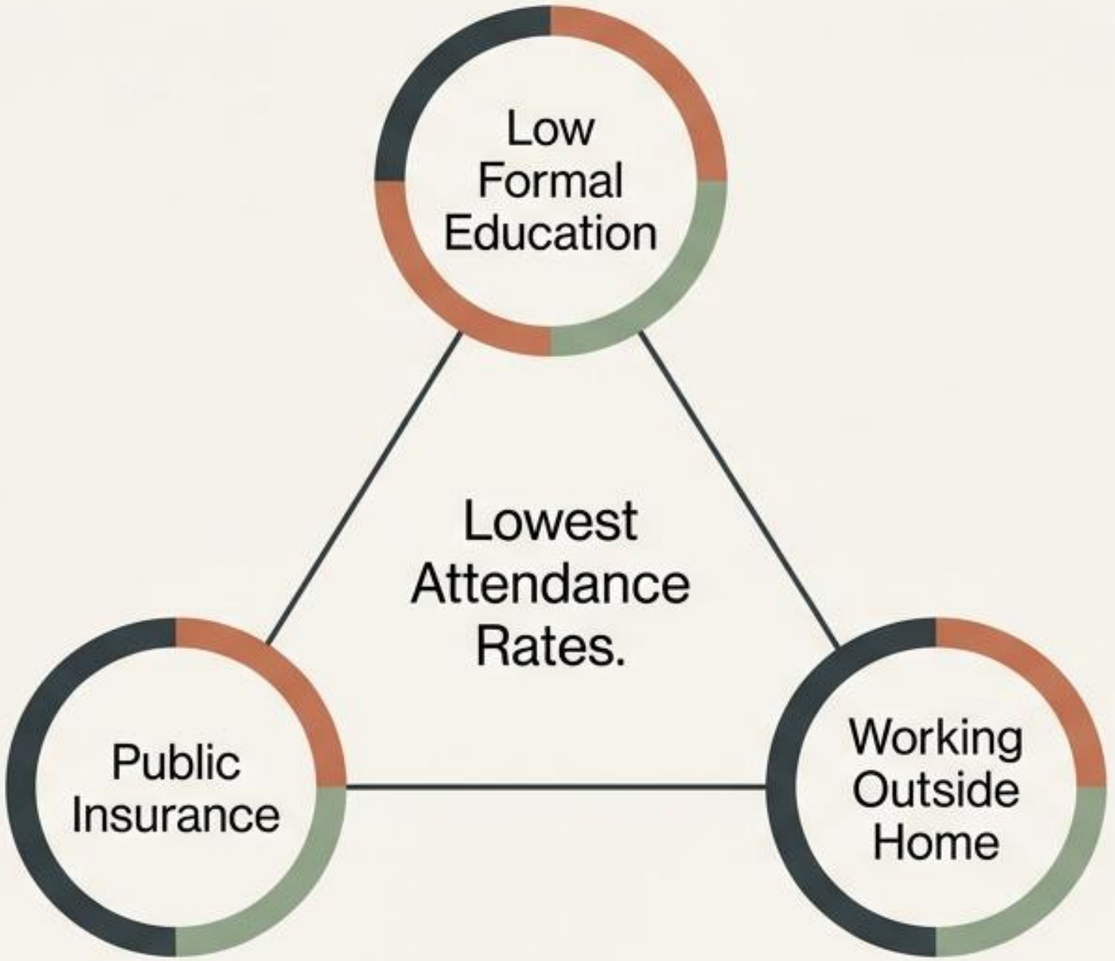
- 4-year college degree (77% attended)
- Private Insurance (77% attended)
- Did not work outside the home (80% attended)

Lower Attendance Likelihood

- Less than a college degree (46% attended)
- Public Insurance (55% attended)
- Working outside the home (56% attended)

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The Web of Factors: Education & Status



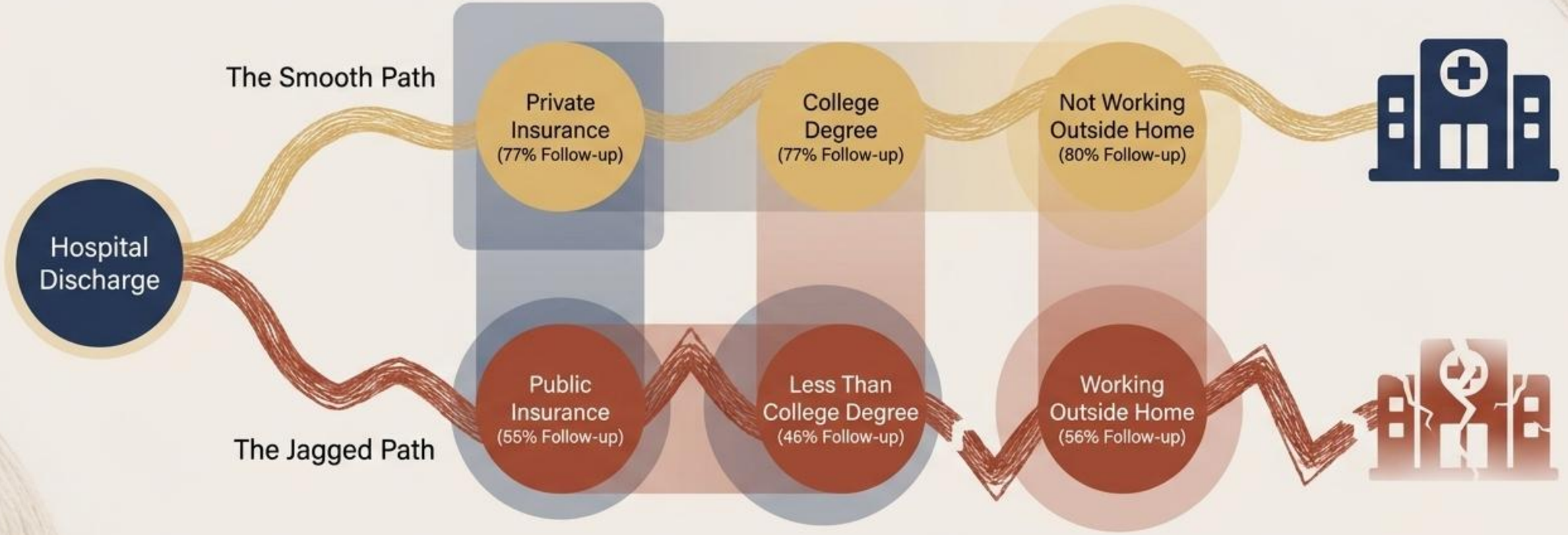
Higher levels of formal education were significantly associated with higher attendance rates ($p < 0.05$).

Low postpartum follow-up is concentrated where these three factors converge, acting as a gatekeeper to survival.

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The Jagged Pathway to Care

Privilege smooths the path; systemic barriers fracture it.



Demographic predictors of postpartum care reveal that attendance is deeply tied to social determinants of health, not just medical need.

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Measuring the Unseen: Stress vs. Agency

JHPCSM
(Contextualized Stress Measure)

What it measures: Chronic intersectional racial and gendered stress.

Root Framework: Critical Race Feminism ("Power-as-control" / Oppressive systems).

Scoring Key:
Higher score = Lower stress levels.

PKPCT
(Power as Knowing Participation)

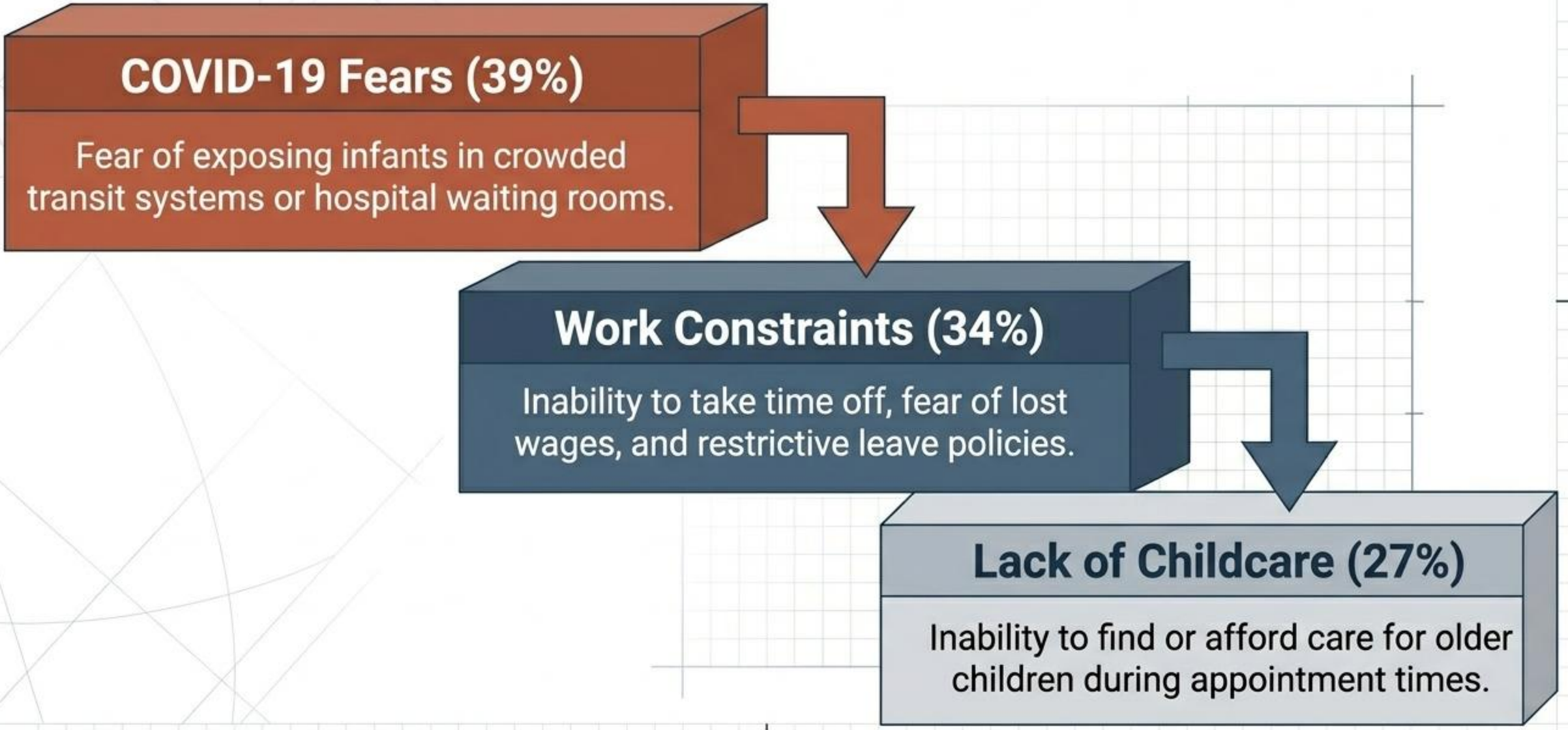
What it measures: Power profiles (awareness, choices, freedom to act intentionally).

Root Framework: Barrett's Theory of Power ("Power-as-freedom" / Personal agency).

Scoring Key:
Higher score = Higher power profile and involvement in change.

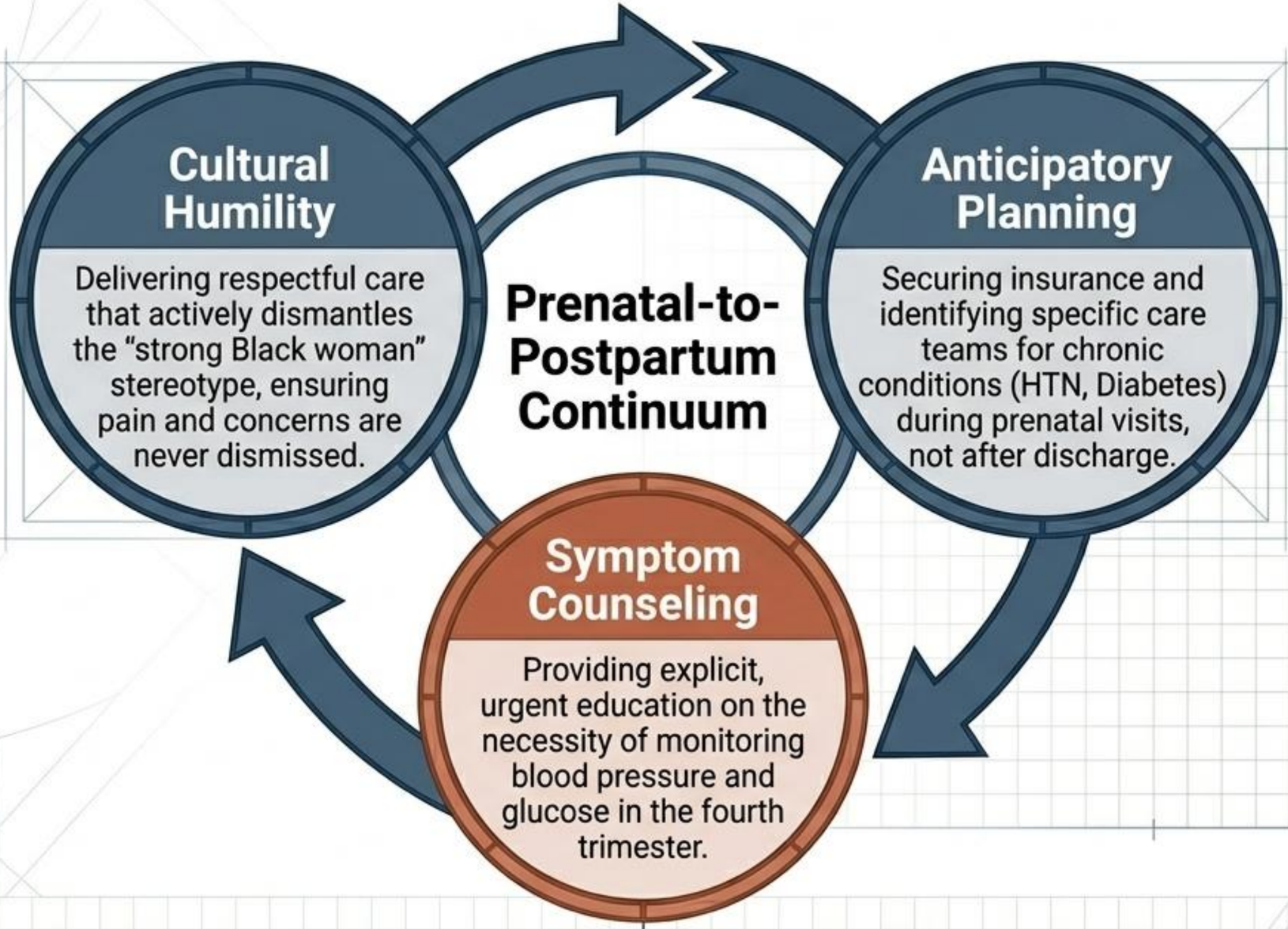
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What Kept Them Away? The Logistical Hurdles



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Implications for Nursing Practice



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Clinical Reform & Nursing Leadership

Action 1: Cultural Humility Training
Goal: Reduce the interpersonal racism and stress measured by the JHPCSM.

Action 2: Quality of Care Improvement
Goal: Recognize the ‘web of factors’ during the postpartum period rather than treating symptoms in isolation.

Action 3: Dismantling Inequities
Goal: Nursing research and practice must actively work to dismantle the structures that cause the 2.5x mortality disparity.



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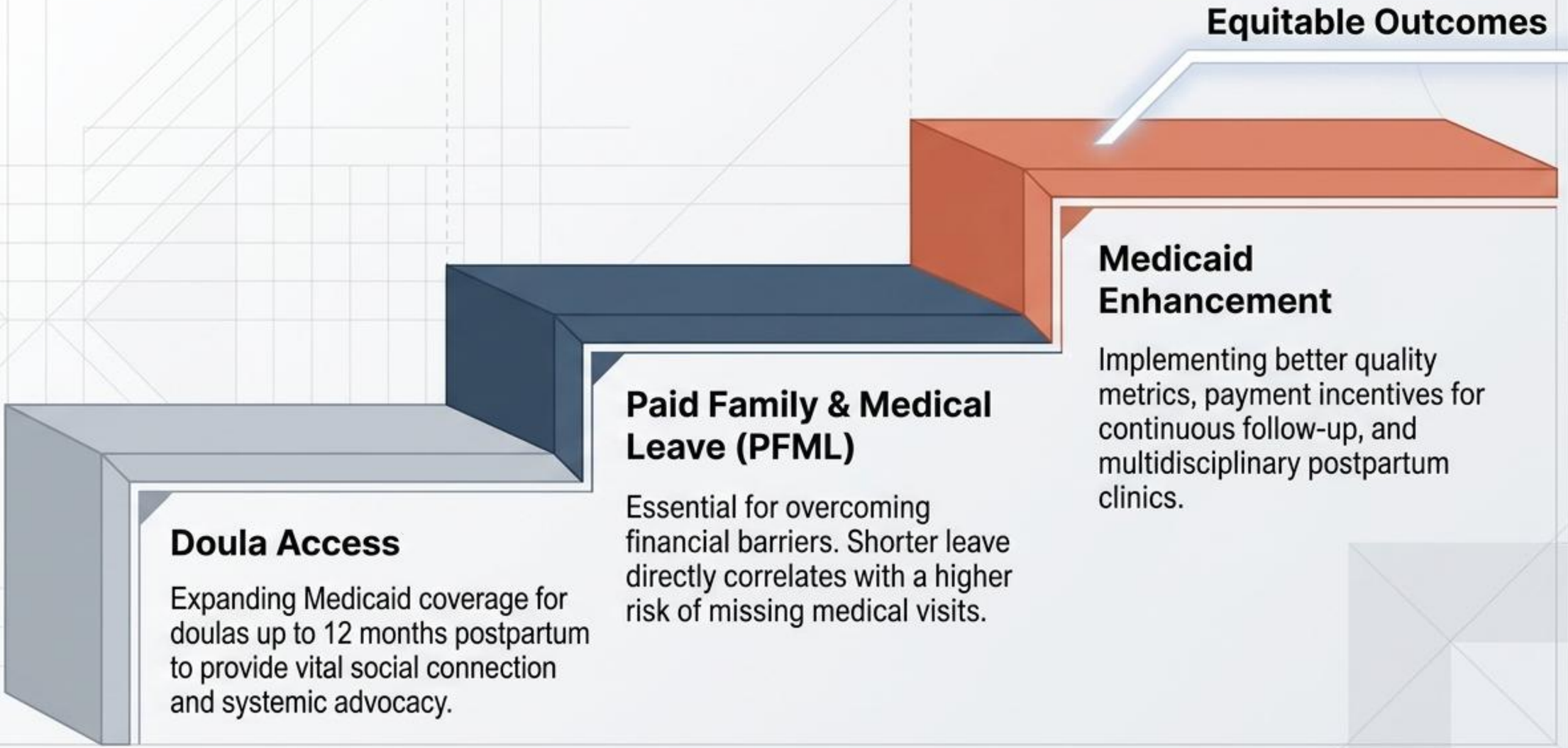
Weaving a New Safety Net

Aligning systemic solutions directly to the identified barriers.

Barrier:	The Bridge:	Policy Mechanism:
Fear of Illness / Clinic Access	De-centralizing Care	Expanded Telehealth & Postpartum Home Visiting Programs.
Inability to take time off work	Economic Protection	Comprehensive Paid Family and Medical Leave (PFML).
Lack of Childcare / Support	Community-Based Advocacy	Medicaid-funded Doula Services up to 12 months postpartum.

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Policy & Structural Solutions



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Momentum for Structural Change

Recent initiatives proving that institutional change is possible.



Massachusetts Maternal Health Bill (Aug 2024)

Mandates insurance coverage for postpartum depression screenings.

Creates a statewide postpartum home visiting service.

Requires MassHealth (Medicaid) coverage for doula services up to 12 months postpartum.



HHS Racial Equity in Postpartum Care Challenge

$1.8 million awarded to ten projects.

Focus on utilizing technology (apps, remote monitoring) to track diabetes and hypertension symptoms postpartum.

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The Core Conclusion

Postpartum health disparities are not the result of individual failings. They are the result of systemic gendered racism.

True birth equity requires shifting the onus of health from the individual to the institution—dismantling the workplace policies, structural hurdles, and clinical biases that marginalize Black birthing people.

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Thank You.

Any questions?

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